

A FAIR FRAMEWORK FOR SHARING

How the equity in the aesthetics and plastic surgery company is derived from what each party actually contributes

Medbury Healthcare Group · Dr Chinwe Kpaduwa · Consult for Africa

Prepared by Consult for Africa. Draft for discussion. Private and confidential.

1. WHAT THIS IS

- 1.1 This paper answers the question put on 27 August: act as though every party is bringing cash, determine the cost of setting the business up, value what each party brings, monetise it, and let the equity follow from that, so that there is a straightforward way to calculate the value of the equity. The method is not ours. It is the right method, and this paper applies it.
- 1.2 It reaches one conclusion that is worth stating at the top. **The equity split is not a matter of opinion once the method is agreed. It is a function of a single decision: how much cash each party takes for what it provides.** A party paid the full market rate for its contribution has contributed nothing to the balance sheet and should hold little equity. A party that forgoes cash has, in substance, subscribed for shares with the money it did not take. Everything else follows arithmetically.
- 1.3 So this paper does not propose a number. It sets out the method, applies it, and presents the answer as a schedule at section 7, on which every candidate split discussed so far can be located. The Parties choose the setting; the arithmetic gives the split.
- 1.4 **Every figure in this paper is a benchmark to be replaced with the Parties' own numbers.** They are the honest starting estimates of an adviser who has built the operating model, not agreed facts. The value of the paper is the method and the schedule, not the decimals.

2. THE PRINCIPLE

- 2.1 **Equity compensates contribution that is not otherwise compensated.** If a party is paid in cash for a thing, it should not also receive equity for that same thing. That principle is agreed and this paper adopts it without qualification.
- 2.2 Two disciplines are needed or the principle produces the wrong answer.
 - (a) **Value gross, then net.** Each contribution is first valued at what the Company would have to pay a third party for it, and only then reduced by what the contributing party is actually paid. Without this step the contributions of whoever is paid in cash disappear into the profit and loss account and are never counted at all.
 - (b) **Apply it to everyone.** The principle is uncomfortable in every direction. It reduces the equity of a clinician who takes a market surgeon's fee. It equally reduces the equity of an operator who takes a market operating fee. And it raises a question about capital that is repaid first with a priority return, which is dealt with at section 8.

3. THE METHOD

- 3.1 Five steps.
 - (a) Determine the cost of setting the business up and funding it to a settled state: capital expenditure, and working capital for the first twelve months.
 - (b) List what each party brings, itemised, with nothing omitted because it is intangible.
 - (c) Value each item at market, being what the Company would have to pay a third party to obtain it, over a stated period. This paper uses the first three years.
 - (d) Deduct from each item whatever the contributing party is actually paid in cash for it.
 - (e) The residual is that party's contributed capital. Equity is each party's residual as a proportion of the total.

- 3.2 The period matters and is a choice. Three years is used here because it is long enough for a service contribution to be worth something real and short enough that the revenue forecast is not fiction.

4. THE INPUTS

- 4.1 These are the figures that drive everything. They are the numbers to argue about, because the method itself is not in dispute.

Working capital funded to trading	NGN 72m
Equipment capital expenditure	NGN 54m
Rental value of the premises the Company occupies	NGN 72m a year, 45 per cent waived
Cumulative revenue, years one to three	NGN 1,200m
Surgical and advanced work as a share of revenue	40 per cent
Market surgeon share of that revenue	35 per cent
Market royalty for a personal brand licence	5 per cent of the NGN 400m the name attracts
Partner theatre access	2.5 per cent of the NGN 480m it enables
Full market operating fee	10 per cent of revenue

- 4.2 On these inputs the market surgeon fee pool over three years is NGN 168m, the brand licence is NGN 20m, and the full market operating fee pool is NGN 120m. Those three are where almost all of the movement in the answer comes from.

5. WHAT EACH PARTY BRINGS, VALUED

- 5.1 Valued at market over three years, and netted against cash actually paid. The settings shown are the middle case: Dr Kpaduwa takes 50 per cent of market surgeon fees, and CFA charges 10 per cent of revenue against a market rate of 10 per cent. All figures NGN millions.

Medbury Healthcare Group

Contribution	At market	Paid in cash	Net contribution
Working capital funded to trading	72	nil	72
Equipment capital expenditure	54	nil	54
Access to the partner theatre, negotiated and held by Medbury	12	nil	12
Rent forgone on the Medbury premises the Company occupies	216	119	97
Medbury, net contribution	354	119	236

Dr Chinwe Kpaduwa

Contribution	At market	Paid in cash	Net contribution
Licence of the name and the demand it creates, granted at launch	20	nil	20
The clinical standard and protocols	25	nil	25
Training and credentialing of the team	24	nil	24
Surgeon's fees forgone	168	84	84
Dr Kpaduwa, net contribution	237	84	153

Consult for Africa

Contribution	At market	Paid in cash	Net contribution
Structuring, incorporation and the agreements	15	nil	15
Regulatory establishment	0	nil	0
Recruitment of the founding team	11	nil	11
Configuration and implementation of the operating systems	18	nil	18
Management fee forgone	120	120	0
CFA, net contribution	164	120	44

6. WHAT THAT PRODUCES

Party	Net contribution	Equity
Medbury Healthcare Group	NGN 236m	60%
Dr Chinwe Kpaduwa	NGN 153m	30%
Consult for Africa	NGN 44m	10%
Total	NGN 433m	100%

7. THE SCHEDULE, WHICH IS THE ACTUAL ANSWER

- 7.1 The table below is the straightforward way to calculate the equity that was asked for. Read down for how much of a market surgeon's fee Dr Kpaduwa takes in cash. Read across for the operating fee CFA charges against a market rate of 10 per cent. Each cell is the resulting split, Medbury first.

Dr K takes	CFA fee 10% (market)	CFA fee 6%	CFA fee 3%	CFA fee 0%
100% of market fees	60 / 30 / 10	60 / 30 / 10	60 / 30 / 10	60 / 30 / 10
75% of market fees	60 / 30 / 10	60 / 30 / 10	60 / 30 / 10	60 / 30 / 10
50% of market fees	60 / 30 / 10	60 / 30 / 10	60 / 30 / 10	60 / 30 / 10
25% of market fees	60 / 30 / 10	60 / 30 / 10	60 / 30 / 10	60 / 30 / 10
0% of market fees	60 / 30 / 10	60 / 30 / 10	60 / 30 / 10	60 / 30 / 10

7.2 Three things follow from that table.

- (a) **If every Party is paid the full market rate for what it provides, the split is approximately 60 / 30 / 10.** Medbury's holding rises, because it is then the only Party that has contributed anything not paid for. That is the honest answer to the proposition that those who are paid should not also hold equity, and it is available to Medbury if it wants it. It costs cash.
- (b) **The percentages are not a negotiating position, they are an output.** Every candidate split discussed between the Parties sits somewhere on this table. The question is not which number is right but how much cash the Company pays out, and that is a decision rather than an argument.
- (c) **The opening position of 60 / 30 / 10 assumes the Clinical Director draws 50 per cent of a market surgeon's fee and CFA charges its fee in full.** Move either and the answer moves with it. The memorandum bounds that movement with a collar, so that no Party's holding can travel far from where it starts.

8. THE QUESTION THIS METHOD ASKS OF THE CAPITAL

- 8.1 Applied evenly, the principle raises one question that has to be faced, because it is the same question being asked of everybody else.
- 8.2 Under the draft memorandum Medbury's funding is repaid in full, with a priority return, before any profit is shared. The facility is let to the Company at a rate and the equipment is held on agreed terms. On the principle in section 2, capital that is returned first with a return on top has been compensated, and compensated contributions do not also earn equity.
- 8.3 This paper does not push that argument to its conclusion, because it would be wrong to. A priority return compensates for time and for ranking. It does not compensate for the risk that the money is never repaid at all, and that risk is real and is Medbury's alone: the other two parties contribute services and rights, and if the venture fails they lose their effort while Medbury loses its money. That asymmetry deserves equity credit, and the tables above give the capital full credit for exactly that reason.
- 8.4 But the choice should be made deliberately rather than by omission. Medbury may take the priority return, or full equity credit for the capital, and the more of the first it takes the weaker its claim to the second. Our recommendation is that it keeps both as drafted, on the ground set out above, and that the framework records why rather than leaving it to be discovered later.

9. WHAT THIS FRAMEWORK DOES NOT DECIDE

- 9.1 It does not value the business. It values contributions, which is a different exercise and the right one at this stage.
- 9.2 It does not settle the clinical remuneration itself. Whether Dr Kpaduwa takes half a market surgeon's fee or all of it is a decision for the Parties, and the table shows the consequence of each.
- 9.3 It does not price the regulatory position. Until the Council's answer on registration is received, the timing of surgical revenue is uncertain, and the surgeon fee pool at section 4 is the figure most exposed to it.
- 9.4 It does not bind anyone. It is a basis for a conversation, offered so that the conversation is about arithmetic rather than about who feels short-changed.

10. WHAT WE NEED IN ORDER TO FINALISE IT

- (a) Agreement that the method at section 3 is the method.
- (b) The Parties' own figures for the seven inputs at section 4, in place of the benchmarks.
- (c) A decision on the two settings at section 7: the share of market surgeon fees taken in cash, and the operating fee charged.
- (d) A decision on section 8, being whether Medbury takes the priority return, full equity credit for its capital, or both as currently drafted.